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Submitted via regulations.gov

Dr. Mehmet Oz
Administrator
Centers for Medicare and Medicaid Services

Dr. Thomas Keane
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U.S. Department of Health and Human Services

Centers for Medicare & Medicaid Services (CMS)
Department of Health and Human Services (HHS)
Attention: CMS–0042–NC, Mail Stop C4–26–05
7500 Security Boulevard
Baltimore, MD 21244–1850

Re: Request for Information; Health Technology Ecosystem (CMS–0042–NC)

Dear Administrator Oz and Assistant Secretary Keane:

Thank you for the opportunity to comment on the Request for Information (RFI) on the Health Technology Ecosystem.

Office Ally is a healthcare technology company that offers cloud-based solutions to healthcare providers, independent physician associations (IPAs) and health plans. Our platform supports both the management of care and facilitates payments between providers, health plans and patients. It is paired with a clearinghouse that enables the secure exchange of healthcare information including claims, remits, and eligibility information between covered entities and across the healthcare market. Office Ally services 1.4 million providers, which represents about a billion claims annually at a total value of $250B in healthcare payments.

Under HHS' and CMS' renewed focus on reducing bureaucratic waste and empowering patients, there is a critical opportunity to elevate the role of clearinghouses in the healthcare system. Rather than serving solely as back-end processors, clearinghouses can become vital drivers of cost savings, faster claims processing, and greater transparency in healthcare transactions. By embracing innovation and real-time data exchange, they can help fulfill the Administration's priorities of streamlining operations, reducing burdens on providers, and putting actionable health information directly in the hands of patients and families. This vision aligns with a broader commitment to restoring trust, accountability, and choice in American healthcare.

Enclosed are additional substantive comments on the RFI. Please do not hesitate to reach out if we can be a resource to you.

Signature,

Chris Hart
CEO

SPECIFIC RFI COMMENTS

B. Patients and Caregivers
1. Patient Needs

PC-1. What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?

Healthcare clearinghouses are well positioned to support digital tools that help patients and caregivers understand and manage their health needs through access to accurate, payer-verified data. Digital health products powered by clearinghouse transactions (such as 270/271 for eligibility and benefits, 837 for claims history, and 278 for prior authorization) can enable several high-impact capabilities.

a) **What are the top things you would like to be able to do for your or your loved ones' health that can be enabled by digital health products?**

The top functionality patients want from digital health tools include: verifying coverage for services in advance of care; identifying in-network providers; reviewing prior claims and remittance details; and tracking eligibility for preventive services. For example, a patient should be able to open an app and immediately see whether a flu shot, colonoscopy, or physical therapy visit is covered and when it's next available—information already contained in 271 responses.

b) **If you had a personal assistant to support your health needs, what are the top things you would ask them to help with? In your response, please consider tasks that could be supported or facilitated by software solutions in the future.**

If these tools functioned like a personal assistant, patients could receive alerts when a new prior authorization is required, get reminders when a service is eligible again, or be guided to the most cost-effective provider based on previous 835 data. Clearinghouses can make this vision real by standardizing and delivering multi-payer administrative data in real-time to patient-facing apps, helping users take informed actions with less friction and more confidence.

PC-2. Do you have easy access to your own and all your loved ones' health information in one location (for example, in a single patient portal or another software system)?

Clearinghouses can aggregate health information across multiple payers, offering a unique path toward a unified patient view. However, most patients and caregivers today do not have easy access to all relevant health data in a single system. Information is often spread across disparate provider portals, creating confusion and administrative burden.

a) **If so, what are some examples of benefits it has provided?**

In cases where partial integration exists—such as access to Medicare claims via Blue Button 2.0 or through a third-party app—patients benefit from visibility into their service history, medication fills, and cost-sharing, which can aid in managing chronic conditions or reconciling bills. This promotes trust and helps prevent unnecessary repeat services.

b) **If not, in what contexts or for what workflows would it be most valuable to use one portal or system to access all such health information?**

Unified access would be especially valuable when coordinating care for aging parents, managing multiple chronic illnesses, or making time-sensitive decisions, such as evaluating a new provider or comparing costs. A single portal that includes eligibility (270/271), claims (837), remittance (835), and prior authorization (278) data would dramatically reduce navigation complexity and enhance decision-making.

c) **Were there particular data types, such as x-rays or specific test results, that were unavailable? What are the obstacles to accessing your own or your loved ones' complete health information electronically and using it for managing health conditions or finding the best care (for example, limitations in functionality, user friendliness, or access to basic technology infrastructure)?**

Key data types such as x-rays, lab results, and care team attribution are often missing or siloed. Common obstacles include limited portal functionality, payer-to-provider data variability, inconsistent formatting of 835 or 278 data, and lack of patient-friendly interfaces. Clearinghouses are positioned to address these challenges by standardizing and delivering complete, structured administrative data that reflects what services were covered, authorized, and paid for—critical insights for managing both health and costs.

PC-4. What features are missing from apps you use or that you are aware of today?

Many current health apps lack the ability to present a complete, payer-verified view of a patient's healthcare experience. Missing features include real-time eligibility status for specific services, integrated prior authorization tracking, explanation of benefits with standardized denial reasons, and consolidated cost-sharing information across payers. These gaps largely stem from limited access to administrative data such as 270/271, 835, and 278 transactions. Clearinghouses already facilitate these transactions at scale and could enable apps to show when a service is next eligible, whether an authorization is pending, and how much a patient is expected to pay—based on actual payer adjudication data. Additionally, few apps support multi-payer data aggregation, which is essential for beneficiaries with both Medicare and supplemental or Medicaid coverage. A clearinghouse-enabled platform could fill these critical gaps, making digital tools significantly more useful for Medicare beneficiaries.

a) **What apps should exist but do not yet? Why do you believe they do not exist yet?**

There is a clear need for an app that provides Medicare beneficiaries with a real-time, multi-payer view of their eligibility, prior authorizations, claims history, and out-of-pocket costs—all in one place. This app should also offer proactive alerts, such as when a preventive service is next eligible or when a claim has been denied. Such an app does not yet exist largely because of the fragmented nature of healthcare data, limited access to real-time payer adjudication information, and the absence of common APIs for core administrative transactions. Clearinghouses already aggregate this data across nearly all payers and could provide the infrastructure for such an application if policy and technical access barriers were addressed.

b) **What set of workflows do you believe CMS is uniquely positioned to offer?**

CMS is uniquely positioned to offer workflows that center around coverage, eligibility, and quality improvement. These include determining next eligible dates for preventive services based on 271 frequency limitations, surfacing pending prior authorizations from 278 data, tracking total cost of care using claims, and offering timely alerts when remittance advice (835) indicates unexpected patient financial responsibility. These workflows rely on standard administrative data that CMS receives today and could be surfaced through patient-facing APIs powered by clearinghouses. This would advance transparency, reduce patient confusion, and support more informed healthcare choices.

PC-5. What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

To increase patient and caregiver interest in digital health products, CMS can leverage its trusted position and vast data assets by enabling more robust, real-time access to administrative health information through consumer-facing applications. Clearinghouses already support bi-directional data exchange with all payers and providers under HIPAA, making them ideal partners in delivering accurate eligibility, claim, and prior authorization data directly to patient apps. CMS can further encourage adoption by developing a "trusted app"

designation tied to data completeness and usability standards. These apps could be featured in Medicare.gov resources or incentivized through Medicare Advantage supplemental benefits. Additionally, CMS should expand Blue Button 2.0 to include 835, 276/277, and 278 transaction data, and allow secure integration of clearinghouse APIs for third-party developers. Clear, plain-language communication campaigns—especially those embedded in provider portals or remittance notices—can raise awareness among caregivers and patients about the tools available to support their care decisions.

PC-6. What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?

To ensure digital health products are accessible to Medicare beneficiaries and caregivers—especially those with limited digital experience—the most important features are simplicity, clarity, and automation. Interfaces should avoid technical jargon and instead present information in plain language, such as "Your next eligible mammogram is covered starting June 1" instead of referencing 271 frequency limits or CPT codes. Visual aids, large fonts, and voice-assisted navigation can further increase usability. Clearinghouse-enabled apps can also reduce user burden by auto-populating forms with verified eligibility and claims data, showing clear explanations of coverage and costs based on 835 data, and surfacing alerts for required actions (e.g., a denied service or a needed authorization). Caregiver proxy access and printable summaries for offline use are also essential. By integrating these features into workflows powered by real-time clearinghouse transactions, CMS can help ensure digital health tools are useful and accessible to the full Medicare population.

PC-7. If CMS were to collect real-world data on digital health products' impact on health outcomes and related costs once they are released into the market, what would be the best means of doing so?

The most effective way for CMS to collect real-world data on the impact of digital health products is by leveraging existing clearinghouse infrastructure to track changes in utilization, cost, and care quality through standard administrative transactions. Claims (837), remittance advice (835), and eligibility (270/271) data can be analyzed pre- and post-adoption to measure outcomes such as preventive service uptake, reduction in denied claims, or fewer unnecessary emergency visits. CMS could also implement event-based triggers—such as a prior authorization submission (278) or service claim—that prompt short patient-reported outcome surveys. These can be distributed through patient portals or partner apps. Additionally, CMS may consider requiring clearinghouses and payers to append a unique digital health product identifier to claims associated with app-enabled interactions, creating a consistent feedback loop. This approach minimizes provider burden and captures impact at scale while respecting existing HIPAA-compliant data flows.

2. Data Access and Integration

PC-8. In your experience, what health data is readily available and valuable to patients or their caregivers or both?

Patients and caregivers most commonly access eligibility (270/271) and claims data (837) through provider or payer portals. These data types are highly valuable for understanding what services are covered, what care has been received, and what financial responsibilities exist. Clearinghouses play a central role in ensuring this data is accurate, current, and available across all payers.

a) **What data is valuable, but hard for patients and caregivers, or app developers and other technical vendors, to access for appropriate and valuable use (for example, claims data, clinical data, encounter notes, operative reports, appointment schedules, prices)?**

Despite its value, data such as prior authorization status (278), remittance advice (835), appointment schedules, and denial reasons (CARCs/RARCs) remains difficult to access consistently. Technical and policy barriers restrict app developers from retrieving this data, even though it is routinely exchanged through

clearinghouses. Clinical documentation such as operative reports or encounter notes is often siloed within EHR systems and not linked to administrative events, making it hard to surface meaningful context in digital tools.

b) What are specific sources, other than claims and clinical data, that would be of highest value, and why?

In addition to clinical and claims data, member attribution rosters (834), preventive service frequency data (within 271), and Z-codes or SDOH indicators are of growing importance. These data points help patients understand who is accountable for their care, what services they're due for, and how social risk factors are influencing their care plans. This is particularly relevant for ACOs and Medicare Advantage plans managing value-based populations.

c) What specific opportunities and challenges exist to improve accessibility, interoperability and integration of clinical data from different sources to enable more meaningful clinical research and generation of actionable evidence?

A major opportunity lies in integrating clearinghouse data with clinical data via FHIR APIs and standardized document formats such as CCDs and C-CDAs. Challenges include aligning data models (X12 vs. FHIR), inconsistent payer implementations, and lack of identity resolution across systems. Clearinghouses can help bridge this gap by matching administrative transactions to clinical events, improving both research data quality and the delivery of actionable insights to patients and caregivers in real time.

PC-9. Given that the Blue Button 2.0 API only includes basic patient demographic, Medicare coverage, and claims data (Part A, B, D), what additional CMS data sources do developers view as most valuable for inclusion in the API to enable more useful digital products for patients and caretakers?

a) What difficulties are there in accessing or utilizing these data sources today?

Today, developers face challenges in accessing this data due to limited API endpoints, lack of real-time updates, and variability in how payers format and release these transaction types. Most 835 and 278 data remain locked in proprietary systems or payer portals, with no standardized patient-facing interface. Clearinghouses already facilitate this exchange at national scale, but there is no current mechanism to expose it via the Blue Button infrastructure.

b) What suggestions do you have to improve the Blue Button 2.0 API experience?

To improve Blue Button 2.0, CMS should expand data availability to include remittance, authorization, and claim status data, with support for event-based updates. Incorporating FHIR-based endpoints that reflect the underlying X12 transactions—supported by clearinghouses—would enable real-time, multi-payer transparency for patients and their apps.

c) Is there non-CMS data that should be included in the API?

Non-CMS data sources that would further enhance the API include Medicare Advantage encounter data, ACO attribution rosters, social risk indicators (e.g., Z-codes or SDOH screenings), and Medicaid dual-eligibility status. These data types are often managed or exchanged through clearinghouses and could be incorporated through trusted intermediary APIs to broaden patient access beyond Fee-for-Service claims alone.

PC-10. How is the Trusted Exchange Framework and Common Agreement (TEFCA) currently helping to advance patient access to health information in the real world?

a) Please provide specific examples.

TEFCA has laid a foundational structure to help patients gain access to their health data, specifically through Individual Access Services (IAS). This is an important step in upholding the patient's right to access and control their health information—one of the cornerstones of the administration's commitment to healthcare transparency and freedom of choice. However, in practice, most patient access still occurs outside of TEFCA—through HIPAA-authorized requests, patient portals, and third-party apps leveraging Cures Act APIs. TEFCA's real-world impact remains limited, especially given slow adoption and lack of incentive alignment.

b) What changes would you suggest?

To advance the administration's priority of reducing regulatory burden and promoting private-sector-led solutions, TEFCA should be limited to core use cases that directly benefit the individual—specifically, treatment and patient-initiated data access. Expanding TEFCA into operational, payment, or public health domains risks duplicating private-sector infrastructure, displacing innovation, and creating costly mandates. Existing systems—particularly clearinghouses—already deliver secure, high-volume data exchange at scale across payers, providers, and other stakeholders. The government should support, not supplant, these trusted and market-proven solutions.

c) What use cases could have a significant impact if implemented through TEFCA?

The most impactful role for TEFCA is to empower individuals and providers:

- **Patients** accessing their complete medical records across networks through simple, secure applications.
- **Doctors** retrieving clinical history from multiple unaffiliated systems in emergency or cross-provider settings.

These use cases fulfill TEFCA's original promise without expanding federal authority over non-clinical data flows.

d) What standards are you aware of that are currently working well to advance access and existing exchange purposes?

Private-sector solutions already rely on widely adopted standards that are functioning effectively:

- **X12 standards** through clearinghouses for administrative data exchange remain foundational and efficient.
- **FHIR APIs**, enabled by the Cures Act, are modernizing patient and payer data access without excessive government control.
- **Direct Secure Messaging** and **IHE-based protocols** continue to support care coordination and referrals.

These standards are interoperable, scalable, and not dependent on federal overreach.

e) Are there redundant standards, protocols, or channels that should be consolidated?

There is increasing risk of duplication and inefficiency. For example, TEFCA's push to create public health reporting pathways overlaps with modernization efforts and existing HL7 channels. Similarly, inserting TEFCA into administrative exchange introduces redundant regulatory layers atop already effective clearinghouse-driven networks. Consolidation and restraint—not expansion—should be the guiding principle.

f) Are there adequate alternatives outside of TEFCA for achieving widespread patient access to their health information?

Yes, and they are working. Under policies championed by President Trump and furthered under current HHS leadership, private innovators have deployed patient-facing apps, interoperable APIs, and low-friction portals. Clearinghouses, health IT vendors, and payer platforms already deliver access, speed, and value—without requiring a new layer of centralized government control. CMS and HHS should continue encouraging these market-led approaches.

PC-11. How are health information exchanges (HIEs) currently helping to advance patient access to health information in the real world?

a) How valuable, available, and accurate do you find the data they share to be?

While HIEs were designed to improve access to longitudinal patient data, the reality has fallen short. Inconsistent participation, limited interoperability, and fragmented governance have restricted their data availability and accuracy across many regions. Much of the data shared via HIEs is still incomplete, out of date, or siloed within limited provider networks. In contrast, clearinghouses already aggregate a more comprehensive and real-time view of patient data by virtue of processing millions of claims, attachments, and medical records—including structured CCDs and FHIR payloads used in prior authorization and clinical documentation. This claim-linked data often reflects the most recent and billable clinical activities across multiple settings and payers, making it more accurate, accessible, and actionable than what is often found in HIE systems.

d) What is the ongoing role of HIEs amidst other entities facilitating data exchange and broader frameworks for data exchange (for example, vendor health information networks, TEFCA, private exchange networks, etc.)?

The role of traditional HIEs should be limited and transitional. In today's data-rich and highly connected environment, multiple private-sector frameworks already outperform legacy HIEs in scope and utility. Vendor-driven networks (such as CommonWell or Carequality), TEFCA's QHINs, and especially clearinghouses, now serve as the real backbone of clinical and administrative interoperability.

Clearinghouses, in particular, occupy a unique position: they are already embedded into nearly every provider and payer workflow, operate under HIPAA-regulated trust frameworks, and process vast quantities of structured, standardized data. Rather than expanding federal or state programs to replicate what the private sector is already doing, CMS and HHS should recognize clearinghouses as natural successors to the HIE model, capable of supporting patient access, care coordination, population health, and even public health reporting—without additional regulatory overhead.

PC-12. What are the most valuable operational health data use cases for patients and caregivers that, if addressed, would create more efficient care navigation or eliminate barriers to competition among providers or both?

Operational health data can meaningfully improve care navigation and promote competition if surfaced to patients in a timely, standardized, and accessible way. Clearinghouses already support many of the foundational data elements needed for this—including eligibility, remittance, and claims-based quality indicators—making them key enablers of these use cases.

a) Examples may include the following:

Examples such as binding cost estimates and provider-specific quality data are increasingly in demand. Eligibility (270/271) and remittance (835) data can support real-time, patient-specific cost estimates, while claims (837) can inform comparative quality metrics. However, provider schedule availability and appointment management require integration with clinical systems, which today operate separately from the clearinghouse infrastructure.

b) What use cases are possible today?

Today, it is feasible to offer pre-service benefit verification, estimates of patient responsibility based on 835 patterns, and retrospective quality indicators based on claims (e.g., preventive service completion). Third-party apps can display in-network providers and cost-sharing details using clearinghouse data.

c) What should be possible in the near future?

In the near future, it should be possible to integrate real-time prior authorization data (278) and payer coverage policy into appointment scheduling tools. Patients could be alerted if a service requires approval, and digital assistants could route them to covered providers with high-quality outcomes.

d) What would be very valuable but may be very hard to achieve?

Achieving true, real-time visibility into provider availability and dynamic scheduling across systems would be highly valuable, but remains difficult due to siloed EHR platforms and a lack of standardized scheduling APIs. Similarly, translating raw clinical outcomes into meaningful, risk-adjusted quality metrics that patients can act on—at the point of decision-making—requires ongoing investment in data standardization, attribution, and usability.

C. Providers
2. Data Exchange

PR-5. Which of the following FHIR APIs and capabilities do you already support or utilize in your provider organization's systems, directly or through an intermediary? For each, describe the transaction model, use case, whether you use individual queries or bulk transactions, and any constraints:

TEFCA is beginning to support provider access to clinical records, but its full potential remains under development. Most current implementations focus on document exchange between provider networks, with limited integration into administrative workflows that directly impact care delivery and operations.

a) **Patient Access API:** While primarily implemented by payers, clearinghouses can enable this API by linking claims and remittance data (837, 835) to patient-authorized apps. The use case is individual-level data retrieval; constraint includes data access authorization across multiple payers.
b) **Standardized API for Patient and Population Services:** Not typically supported directly by clearinghouses, but they can enhance this with bulk eligibility checks or claims-based quality metrics at the population level.
c) **Provider Directory API:** Clearinghouses often maintain extensive provider network directories through trading partner relationships and can supply validated data for this API. This supports both individual queries and batch directory updates.
d) **Provider Access API:** Clearinghouses can enable provider access APIs by linking active patient claims to attribution data and supporting verification of treatment relationships—essential for ACOs and MA plans.
e) **Payer-to-Payer API:** Clearinghouses often act as intermediaries in payer-to-payer transitions, particularly for eligibility (834) and historical claims data. Use case is individual or batch transmission of historical care data when members switch plans.
f) **Prior Authorization API:** This is an emerging function. Clearinghouses currently support X12 278 transactions and can serve as a foundation for FHIR-based Prior Auth APIs (PAS). Constraints include varying levels of EHR integration with authorization workflows.
g) **Bulk FHIR:** Some clearinghouses support group-level batch queries through their existing EDI infrastructure, particularly for eligibility and claims history. While not fully FHIR-based, this model could be extended to meet the use case.

h) **SMART on FHIR:** Deployment is typically handled by provider organizations. However, clearinghouses could support backend data feeds or integration for claims and eligibility information that would be accessed via SMART apps.
i) **CDS Hooks**: Not directly supported, but clearinghouses can supply data inputs—such as authorization requirements or recent denial patterns—that could trigger clinical decision support logic when integrated through EHR vendors.

PR-7. What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem? How can the burden of increasing data availability and sharing be mitigated for providers? Are there ways that workflows or metrics that providers are already motivated to optimize for that could be reused for, or combined with, efforts needed to support interoperability?

CMS can support providers in improving data quality and interoperability by aligning existing reporting and administrative workflows with digital health data-sharing requirements. Many providers already submit standardized data to clearinghouses for eligibility (270/271), claims (837), and remittance (835). Leveraging this infrastructure to pre-populate or validate data for digital products reduces duplication and ensures accuracy. To reduce provider burden, CMS could allow clearinghouse-submitted data to serve dual purposes—for example, using 837 claims to meet both payment and quality measurement requirements, or allowing 270/271 transactions to support care coordination and eligibility verification in digital tools. Additionally, CMS should invest in automation and bulk FHIR APIs that reduce manual chart abstraction, encourage adoption of CDS Hooks and SMART on FHIR apps that deliver interoperability within the clinical workflow, and promote data standards alignment across EHRs and clearinghouses. Real-time authorization data, patient attribution feeds, and service utilization summaries could all be surfaced from existing clearinghouse data flows with minimal provider effort, advancing interoperability without imposing new documentation burdens.

PR-8. What are ways CMS or partners can help with simplifying clinical quality data responsibilities of providers?

CMS can simplify clinical quality data responsibilities by aligning reporting requirements with the data providers already submit through clearinghouses and EHRs. Much of the information needed for quality measurement—such as diagnoses, procedures, and preventive services—is already present in standard administrative transactions such as the 837 (claims), 835 (remittance), and 270/271 (eligibility). By allowing this data to be reused for digital quality measurement, CMS can reduce redundant documentation and manual chart review.

a) **What would be the benefits and downsides of using Bulk FHIR data exports from EHRs to CMS to simplify clinical quality data submissions? Can CMS reduce the burden on providers by performing quality metrics calculations leveraging Bulk FHIR data exports?**

Using Bulk FHIR exports from EHRs to CMS can streamline quality reporting by allowing CMS to perform measure calculations centrally, reducing the need for each provider to manage its own quality engine. This would lower technical overhead, ensure consistency in interpretation, and help smaller or rural providers participate in programs they might otherwise avoid. However, risks include incomplete EHR data capture, potential latency in batch updates, and reduced transparency if providers lack visibility into how quality scores are derived. CMS can mitigate these risks by providing real-time dashboards and feedback loops, and by supplementing FHIR data with clearinghouse-derived claims and remittance information.

b) **In what ways can the interoperability and quality reporting responsibilities of providers be consolidated so investments can be dually purposed?**

Interoperability and quality reporting should be designed around shared data pipelines. Claims (837), eligibility (270/271), and remittance (835) data already submitted through clearinghouses can serve dual purposes: both

for payment operations and for quality tracking. Aligning data submission requirements for MIPS, HEDIS, and ACO programs around common formats (FHIR, X12, QRDA) and data definitions would maximize the return on providers' existing reporting infrastructure.

c) **Are there requirements CMS should consider for data registries to support digital quality measurement in a more efficient manner? Are there requirements CMS should consider for data registries that would support access to real-time quality data for healthcare providers to inform clinical care in addition to simplifying reporting processes?**

CMS should ensure that registries certified for digital quality measurement can ingest both clinical (EHR-derived) and administrative (clearinghouse-derived) data. Requirements should include support for HL7 FHIR, standardized APIs, and timely bi-directional exchange. Registries should also surface actionable insights—such as overdue screenings or gaps in risk coding—within the provider's workflow. Real-time registry feedback, built on claims and EHR integration, would both improve patient care and reduce retrospective audit burdens. Where feasible, CMS should incentivize alignment between registry functions and existing clearinghouse services to avoid redundant systems and streamline provider participation.

D. Payers

PA-1. What policy or technical limitations do you see in TEFCA? What changes would you suggest to address those limitations? To what degree do you expect these limitations to hinder participation in TEFCA?

TEFCA's current focus on clinical document exchange limits its utility for payers, who rely heavily on administrative data such as eligibility (270/271), claims (837), remittance (835), and prior authorization (278) to manage care and risk. These transactions are not yet integrated into TEFCA, making it less relevant to core payer workflows and reducing the incentive for participation. Additionally, TEFCA lacks clear alignment with existing clearinghouse infrastructure, which already connects all major payers and providers through standardized X12 transactions. Without support for these workflows, TEFCA risks duplicating efforts rather than enhancing interoperability. Additionally, there is a current lack of alignment on how to securely exchange sensitive data via TEFCA. In particular, how does one know the entity asking for data has the proper permission and valid use case under HIPAA to have said data? Clearinghouses have already addressed this issue over years of use and have processes and procedures in place to validate both parties.

PA-2. How can CMS encourage payers to accelerate the implementation and utilization of APIs for patients, providers, and other payers, similar to the Blue Button 2.0 and Data at the Point of Care APIs released by CMS?

CMS can encourage payers to accelerate API adoption by aligning incentives with operational priorities and leveraging existing clearinghouse infrastructure to minimize implementation burden. Specifically, CMS could tie Medicare Advantage Star Ratings, risk adjustment accuracy, or participation in value-based programs to payer API readiness and usage metrics. To drive adoption, CMS should allow clearinghouses to serve as intermediaries for payers, enabling compliant FHIR APIs that expose existing X12 transactions—such as eligibility (270/271), claims (837), and prior authorization (278)—through modern interfaces. This would give patients, providers, and other payers real-time access to actionable information without requiring each payer to build and maintain standalone systems.

PA-5. What are ways payers can help with simplifying clinical quality data responsibilities of providers?

Payers can simplify clinical quality data responsibilities by leveraging data already exchanged through healthcare clearinghouses. Clearinghouses process standardized administrative transactions—such as claims (837), remittance advice (835), and eligibility (270/271)—that contain much of the information needed for quality measurement. Rather than requiring separate chart extractions or manual submissions, payers can

use this existing data to calculate metrics related to preventive screenings, chronic disease management, and medication adherence. For example, a mammogram billed on an 837 claim or a denial reason coded on an 835 can indicate whether a quality measure was met or missed. Payers can use these insights to pre-populate reporting dashboards or notify providers of open gaps without requiring additional provider input. This reduces documentation burden while maintaining accuracy and timeliness. By integrating clearinghouse data into quality reporting workflows and aligning those measures with CMS programs, payers can create a streamlined, low-friction process that supports providers without adding new requirements. This approach not only lowers cost and complexity but also promotes consistent, real-time quality improvement across the healthcare system.

a) **How interested are payers and providers in EHR technology advances that enable bulk extraction of clinical quality data from EHRs to payers to allow them to do the calculations instead of the provider-side technology?**

Payers recognize that combining bulk clinical data with claims, remittance, and eligibility data already processed through clearinghouses creates a more complete and reliable dataset for quality measurement. This hybrid approach allows payers to calculate metrics without placing additional technology or compliance burdens on providers.

b) **In what ways can the interoperability and quality reporting responsibilities of providers to both CMS and other payers be consolidated so investments can be dually purposed? Are there technologies payers might leverage that would support access to real time quality data for healthcare providers to inform clinical care in addition to simplifying reporting processes?**

Interoperability and quality reporting responsibilities can be consolidated by reusing data already exchanged through clearinghouses to satisfy requirements across both CMS and commercial payers. Providers already submit eligibility (270/271), claims (837), and remittance (835) data that contain key quality indicators—such as diagnosis codes, screening services, and patient financial barriers. Aligning CMS and payer quality programs to accept these standard transactions as a primary data source would allow providers to meet multiple reporting requirements with a single data stream. Payers can also leverage clearinghouse infrastructure to deliver real-time feedback to providers. For example, integrating claim-based care gap alerts into EHRs via APIs or dashboards enables clinicians to address missing quality actions during patient visits. When paired with FHIR-based clinical data from EHRs, this creates a unified view of quality performance that supports both care delivery and reporting.

E. Technology Vendors, Data Providers, and Networks
1. Ecosystem

TD-1. What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

In the short term, CMS can stimulate developer interest by expanding access to high-value, real-time administrative data already available through clearinghouses. This includes eligibility (270/271), claims (837), remittance (835), and prior authorization (278) transactions. Making these datasets available through standardized APIs—modeled on Blue Button 2.0—would give developers a strong foundation to build tools that help Medicare beneficiaries manage costs, navigate care, and track service history. CMS can also launch a "trusted data intermediary" program that allows clearinghouses to serve as authorized data access partners for developers, reducing integration complexity and speeding time to market. Offering developer grants, challenge competitions, and regulatory guidance aligned with HIPAA and FHIR standards would further reduce barriers to entry.

TD-2. Regarding CMS Data, to stimulate developer interest—

a) **What additional data would be most valuable if made available through CMS APIs?**

Attributed provider or care team information (834/rosters): Useful for value-based care management, helping patients know who is responsible for coordinating their care.

b) What obstacles prevent accessing these data sources today?

Fragmented data ownership and formats: Data is often siloed across CMS, MA plans, and clearinghouses, with inconsistent formats (X12, FHIR, proprietary). This makes integration difficult for developers without standardized, unified access points.

c) What other APIs should CMS and ASTP/ONC consider including in program policies to unleash innovation and support patients and providers?

Remittance Advice API (835): Allows patients and providers to view payer adjudication results, denials, and financial responsibility, enabling transparent billing tools and real-time revenue cycle tracking.

Prior Authorization API (278 + FHIR PAS): Supports automated submission and status tracking of prior auth requests, reducing delays in care and provider administrative burden.

Claim Status API (276/277): Enables near real-time updates on submitted claims, helping providers and patients quickly identify issues or pending actions.

Preventive Service Eligibility API (enhanced 270/271): Helps developers surface personalized wellness alerts and eligibility for screenings based on benefit frequency limits.

Attribution and Roster API (834): Enables value-based care tools to track which patients are assigned to which providers or care teams, essential for care coordination and shared savings models.

Patient Event Notification API (ADT/FHIR Encounter): Facilitates real-time alerts when patients are admitted, discharged, or transferred—critical for reducing readmissions and managing transitions of care.

3. Technical Standards and Certification

TD-4. How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?

CMS can accelerate adoption of open, standards-based, publicly available APIs by expanding the role of healthcare clearinghouses as API infrastructure providers. Clearinghouses already operate at national scale, exchanging X12 transactions—such as eligibility (270/271), claims (837), remittance (835), and prior authorization (278)—across nearly all payers and providers. This uniquely positions them to serve as the operational backbone for exposing these data sets via FHIR-based APIs. To encourage this shift, CMS should formally recognize clearinghouses as trusted API intermediaries that can transform existing HIPAA-standard transactions into patient- and provider-facing APIs. This approach avoids duplicating infrastructure, reduces compliance complexity for smaller entities, and ensures that API adoption is grounded in workflows already in widespread use.

TD-7. To what degree has USCDI improved interoperability and exchange and what are its limitations?

USCDI has improved interoperability by standardizing a core set of clinical data elements for exchange, helping align EHR systems around a common structure. It has enabled more consistent clinical data sharing through FHIR APIs and TEFCA participation. However, USCDI remains limited in scope for administrative and

operational use cases that are critical to patients, payers, and providers—areas where clearinghouses have mature infrastructure and transactional standards.

TD-9. Regarding certification of health IT:

Reimagining and expanding the role of clearinghouses offers CMS a rapid, scalable path to advancing health IT certification goals without requiring a complete reinvention of infrastructure. Clearinghouses already serve as trusted intermediaries under HIPAA, processing and standardizing transactions across all payers and providers. By building upon this existing foundation, CMS and ONC can accelerate interoperability and data liquidity.

a) **What are the benefits of redefining certification to prioritize API-enabled capabilities over software functionality?**

Redefining certification to prioritize API-enabled capabilities over static software functionality allows CMS to drive real-time, standards-based data exchange. Clearinghouses can serve as backend API hubs, exposing eligibility (270/271), claims (837), remittance (835), and prior authorization (278) data through secure, FHIR-compliant interfaces. This approach accelerates adoption, reduces custom builds, and leverages a network already operating at national scale.

b) **What would be the drawbacks?**

The primary drawback is that many EHR vendors and small providers may not have direct API capabilities for administrative data or unstructured content. Without leveraging clearinghouses as intermediaries, this shift could disproportionately burden providers with limited IT resources.

c) **How could ASTP/ONC revise health IT certification criteria to require APIs to consistently support exchanging data from all aspects of the patient's chart (for example, faxed records, free text, discrete data)?**

ASTP/ONC could revise certification criteria to require APIs that support exchange of all patient-relevant data—including scanned documents, PDFs, free-text notes, and structured fields—by encouraging hybrid models. Clearinghouses could act as data routers or converters, translating these inputs into standard payloads for downstream use, minimizing provider burden.

d) **What policy changes could CMS make so providers are motivated to respond to API-based data requests with best possible coverage and quality of data?**

CMS can motivate providers to respond to API-based data requests by aligning reimbursement and quality incentives with API utilization. For example, providers could earn MIPS or ACO performance credit for responding to payer or patient data requests via certified APIs. CMS should also permit clearinghouses to mediate these exchanges, allowing providers to fulfill data requests through existing workflows and infrastructure.

e) **How could EHRs capable of bulk data transfer be used to reduce the burden on providers for reporting quality performance data to CMS? What capabilities are needed to show benefit? What concerns are there with this approach?**

EHRs capable of bulk data transfer—especially when paired with clearinghouse administrative feeds—can significantly reduce provider reporting burden. CMS can ingest quality-relevant data directly, calculate measures centrally, and return actionable performance insights. To demonstrate value, capabilities should include patient-matching, data completeness validation, and alignment with clearinghouse claims data for cross-validation. Concerns include data governance, consent management, and provider visibility into how

their data is being interpreted—challenges that clearinghouses are well-equipped to manage securely and at scale.

TD-10. For EHR and other developers subject to the ONC Health IT Certification Program, what further steps should ASTP/ONC consider to implement the 21st Century Cures Act's API condition of certification (42 U.S.C. 300jj-11(c)(5)(D)(iv)) that requires a developer's APIs to allow health information to be accessed, exchanged, and used without special effort, including providing access to all data elements of a patient's electronic health record to the extent permissible under applicable privacy laws?

To fully implement the 21st Century Cures Act's API condition of certification, ASTP/ONC should take steps that align with existing clearinghouse infrastructure to ensure access, exchange, and use of health information "without special effort." Clearinghouses already enable seamless, standardized exchange of HIPAA-regulated administrative data across nearly all payers and providers. By expanding their role, CMS can move quickly to operationalize the Cures Act's intent. ONC should require that certified APIs not only support clinical EHR data (structured notes, labs, medications), but also integrate key administrative data—such as eligibility, claims, prior authorization, and remittance—using existing standards such as X12 and mapped FHIR equivalents. These data types are essential for care coordination, patient financial transparency, and value-based program success.

4. Data Exchange

TD-12. Should CMS endorse non-CMS data sources and networks, and if so, what criteria or metrics should CMS consider?

Yes, CMS should endorse non-CMS data sources and networks—especially healthcare clearinghouses—because they already facilitate trusted, bi-directional data exchange between all major payers and providers across the U.S. healthcare system. Endorsing clearinghouses would allow CMS to rapidly scale access to high-value administrative data and complement its clinical data initiatives under TEFCA. To ensure quality and alignment with CMS objectives, the following criteria and metrics should guide endorsement: Standards compliance: The network must support HIPAA-mandated X12 transactions (270/271, 837, 835, 278, etc.) and/or FHIR-based APIs aligned with CMS interoperability rules. National payer and provider coverage: Endorsed networks should demonstrate broad connectivity across Medicare, Medicaid, commercial plans, and provider organizations to ensure completeness and utility of exchanged data. Data accuracy and timeliness: The network must provide validated, real-time or near-real-time access to administrative data, such as eligibility verification, claim status, and prior authorization outcomes. Security and privacy compliance: Networks must meet HIPAA security standards and offer mechanisms for identity verification, patient consent, and auditability.

TD-13. What new opportunities and advancements could emerge with APIs providing access to the entirety of a patient's electronic health information (EHI)?

Clearinghouses, which already manage comprehensive administrative datasets across all payers, are ideally positioned to help operationalize full-EHI access by combining their existing transaction streams with expanding API capabilities. New opportunities include: Real-time cost and coverage visibility: By integrating claims (837), remittance (835), and eligibility (270/271) data into EHI-accessible APIs, patients and providers could receive accurate, up-to-date information on coverage, patient responsibility, and benefit limits—enabling smarter care decisions and reducing surprise billing. Seamless prior authorization workflows: Access to 278 prior authorization data through standardized APIs would allow digital health tools and EHRs to automate authorization tracking, reducing delays and provider burden. Integrated quality measurement and care gap alerts: When combined with clinical data from EHRs, clearinghouse data can support automated quality reporting and real-time notifications for open care gaps or missed diagnoses, especially in value-based care settings. Cross-payer, longitudinal health records: Clearinghouse-enabled APIs can unify data across Medicare, Medicaid, and commercial plans, giving patients and care teams a holistic view of the care journey

regardless of plan transitions. Better data for analytics and equity: Full-EHI APIs that include administrative detail can improve population health analytics, social risk adjustment, and benchmarking efforts by enabling richer, multi-dimensional datasets.

a) **What are the primary obstacles to this?**

Data fragmentation: Clinical data is often siloed across EHR systems, while administrative data resides with payers and clearinghouses. Without unified APIs or shared governance, accessing a complete EHI requires stitching together disparate sources.

Lack of infrastructure integration: EHR vendors typically do not expose administrative data through their APIs, and many API certification efforts exclude claims, eligibility, and prior auth—despite their availability through clearinghouses.

Inconsistent standards: Clinical data flows through FHIR and C-CDA, while administrative data follows X12 formats. Without reliable mapping between these standards, full EHI APIs face implementation friction.

Privacy and authorization complexity: Managing patient consent across multiple data sources, especially for sensitive or longitudinal data, presents logistical and regulatory challenges that clearinghouses are well-equipped to help manage but are not yet fully empowered to address under current API frameworks.

b) **What are the primary tradeoffs between USCDI and full EHI, especially given more flexible data processing capabilities today?**

USCDI strengths: It provides a consistent, structured clinical dataset with known semantics and broad EHR vendor support, making it well-suited for interoperability and initial data access use cases.

USCDI limitations: It excludes critical administrative data—such as financial responsibility, prior auth status, and payer attribution—that are essential to patient decision-making and care management. It also omits unstructured content such as scanned documents or attachments used in adjudication workflows.

Full EHI value: Full EHI access, particularly when enriched with clearinghouse-supported administrative data, enables richer analytics, end-to-end care visibility, and more meaningful support for value-based care. It also allows the inclusion of real-world data inputs (e.g., prior denials, claims timing, or eligibility events) that USCDI alone does not capture.

Tradeoff: While full EHI may require more sophisticated data processing (e.g., parsing PDFs, mapping X12 to FHIR), the gain in completeness and actionable insight outweighs the complexity—especially given recent advances in language models and natural language processing. To address both challenges and tradeoffs, CMS should support hybrid models that use USCDI as a foundation and expand full EHI APIs with clearinghouse-enabled administrative datasets for a truly comprehensive health information view.

TD-14. Regarding networks' use of FHIR APIs:

a) **How many endpoints is your network connected to for patient data sharing? What types, categories, geographies of endpoints do you cover? Are they searchable by National Provider Identifier (NPI) or organizational ID?**

More than 80,000 healthcare organizations exchange more than 1 billion transactions annually across Office Ally's clearinghouses. Through existing trading partner relationships with other clearinghouses, we can connect every provider with every payer.

b) **How are these connections established (for example, FHIR (g)(10) endpoints, TEFCA/Integrating the Health Enterprise (IHE) XCA, or proprietary APIs)?**

With Providers: Enrollment & Trading Partner Agreement (TPA): Providers sign a TPA with the clearinghouse, authorizing it to send and receive HIPAA transactions (e.g., 837 claims, 270 eligibility requests) on their behalf.

Practice Management or EHR Integration: The clearinghouse integrates with the provider's practice management system (PMS) or EHR, either through direct API connections, secure file transfers, or embedded modules.

Submitter ID & Testing: The provider is issued a unique ID and completes testing to ensure transaction formats (X12) and data mappings are valid. This step confirms the provider's system can communicate reliably with the clearinghouse.

Ongoing Support: Clearinghouses offer tools for claims validation, edits, and rejections, as well as portals for eligibility, claim status (276/277), and remittance (835) lookup.

With Payers: Payer Connectivity Agreement: Clearinghouses establish contractual agreements with each payer, defining the types of transactions exchanged, data routing rules, and security expectations.

EDI Enrollment: Providers must also enroll with the payer (often facilitated by the clearinghouse) to authorize that clearinghouse to exchange EDI data on their behalf.

Connectivity Method: Data exchange with payers occurs via: Secure FTP or HTTPS connections Real-time APIs (for eligibility or claim status) Mailbox polling or VAN (Value-Added Network) services.

Clearinghouse Payer ID: Each payer is identified by a unique "Payer ID" used by the clearinghouse to route transactions correctly.

c) **Do you interconnect with other networks? Under what frameworks (for example, TEFCA, private agreements)?**

We have bi-directional data sharing agreements with other clearinghouses.

TD-16. What are the tradeoffs of maintaining point-to-point models vs. shared network infrastructure?

Maintaining point-to-point data exchange models creates significant inefficiencies, fragmentation, and duplication across the healthcare system. Each payer, provider, and vendor must manage separate connections, formats, testing, and maintenance—resulting in high costs, slow onboarding, and inconsistent data exchange. In contrast, the existing clearinghouse infrastructure offers a proven, scalable shared network model that connects virtually all payers and providers under a unified framework based on HIPAA-standard transactions. Clearinghouses have already established trusted, bi-directional connections with thousands of trading partners. They manage enrollment, routing, validation, and compliance across eligibility (270/271), claims (837), remittance (835), prior authorization (278), and more—enabling seamless, secure interoperability without the need for duplicative point-to-point integrations.

a) **Do current rules encourage scalable network participation?**

Current rules, while supportive of open APIs, still allow and often necessitate siloed, proprietary connections—particularly for prior authorization and payer-to-provider communication. This discourages shared network participation and creates fragmentation. There is no mandate to leverage existing clearinghouse networks for API enablement, which overlooks their infrastructure readiness and broad connectivity.

b) What changes would improve alignment (for example, API unification, reciprocal access)?

To improve alignment, CMS should explicitly support shared clearinghouse infrastructure as a designated interoperability network for administrative data, similar to TEFCA for clinical data. This would include: Mandating API unification for common transactions (e.g., transforming 278 into FHIR PAS using clearinghouse intermediaries); Requiring reciprocal access between payers, providers, and third-party apps through certified clearinghouses; Standardizing credentialing and enrollment processes to reduce barriers to data sharing; Offering regulatory recognition or endorsement of clearinghouses as qualified health information networks for administrative interoperability.

TD-17. Given operational costs, what role should CMS or ASTP/ONC or both have in ensuring viability of healthcare data sharing networks, including enough supply and demand, that results in usage and outcomes?

CMS and ASTP/ONC should play a direct role in ensuring the long-term viability of healthcare data sharing networks by creating dedicated incentive programs for clearinghouses to expand their data exchange capabilities. Clearinghouses already operate the most robust, nationwide infrastructure for administrative data exchange—supporting HIPAA-standard transactions across virtually all payers and providers. However, enabling real-time APIs, expanded data formats (e.g., FHIR), and integration with clinical systems involves significant upfront investment and ongoing operational cost. To accelerate adoption and ensure sufficient supply and demand for interoperable data exchange, CMS should allocate incentive dollars—similar to the EHR Meaningful Use program—to clearinghouses that: Develop and expose open APIs for claims, eligibility, remittance, and prior authorization. Support digital quality measurement and real-time care coordination through integrated administrative data feeds. Enable patient-directed data sharing with certified apps, including Medicare beneficiaries. Integrate clearinghouse administrative workflows with TEFCA and FHIR-based clinical networks. Incentivizing clearinghouses ensures high-impact, rapid deployment of scalable data sharing services. It also avoids duplicative infrastructure spending by leveraging trusted networks already in place. By investing in the backbone of healthcare's administrative infrastructure, CMS can ensure that data exchange leads to measurable outcomes—such as reduced delays, improved quality reporting, and more informed patient decision-making—at national scale.

5. Compliance

TD-19. Regarding price transparency implementation:

Price transparency remains limited by inconsistent data formats, delayed delivery, and lack of integration into real-time workflows. Most provider-facing tools rely on payer portals or static machine-readable files that are difficult to interpret and not actionable at the point of care. As a result, price estimates are often unavailable when patients and providers are making key decisions.

a) What are current shortcomings in content, format, delivery, and timeliness?

Current shortcomings include lack of patient-specific estimates, fragmented delivery through non-standard portals, and the absence of pricing information in core transactional workflows. There is no unified standard for delivering expected costs tied to benefits, service codes, or network status.

b) Which workflows would benefit most from functional price transparency?

The workflows that would benefit most from functional price transparency include eligibility verification, scheduling, pre-service estimates, and prior authorization. Embedding real-time pricing into eligibility

transactions (270/271) would provide immediate value to providers, digital tools, and patients during appointment booking and intake.

c) What improvements would be most valuable for patients, providers, or payers, including CMS?

The most valuable improvement would be for payers to include estimated allowed amounts, patient responsibility, and remaining deductible or out-of-pocket information in the 271 response. Clearinghouses already support eligibility verification at scale and could route or enrich these transactions with real-time pricing data from payer systems, creating a seamless cost estimate workflow.

d) What would further motivate solution development?

To further motivate solution development, CMS should: Establish implementation guidance and pilot programs for pricing data within 271 transactions; Incentivize payers and clearinghouses to support enriched eligibility responses through payment or regulatory credits; Align price transparency compliance with CMS reimbursement incentives, such as Medicare Advantage Star Ratings or value-based care participation.